The Hillingdon Hospitals
NHS Foundation Trust

Intrauterine Death, Neonatal Death and Late Termination of Pregnancy: Medical Management

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6.0	Shabana Syed/ Sameera Tanna	• Minor amendments to layout/formatting, instructions from P39 onwards
7.0	Leila Abdou	• Amendments to include changes to use of electronic records • Checklists updated
7.1	Leila Abdou	• Amendments to appendices to include funeral arrangments

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Contents

Aims 4
Introduction 4
The Guidance 4
Initial management of pregnancy loss 4
Initial management of a fetocide (>21 weeks gestation) 5
Timing of delivery 5
Process For Delivery 5
Care Pathway 5
Consent to Process 6
Pain Relief in Labour 6
Inducing Labour and Delivery 6
Preparation for Delivery 7
Delivery 8
Management of the third stage 8
Care of the Baby's Body 9
Preparing the baby for viewing: 9
Taking hand and footprints: 9
Taking Photographs 9
Blessings/Saying Goodbye 9
Advice for Parents Choosing a Postmortem 10
Parents who are undecided or who have declined the postmortem 11
Placenta Only 11
Referral for Cytogenetics Analysis of Amniotic Fluid, CVS and Solid Tissue Samples 11
The Hillingdon Hospital investigations 11
Prepare the baby for transfer to the mortuary 11
Documents required to go with the baby to the mortuary <24/40 12
Documents required to go with the baby to the mortuary >24/40 or <if registered alive birth 12
Funeral Arrangements 12
Neonatal Death 13
Discharge 13
Postnatal visits: 13
Associated Guidelines, Policies and Documents 14
Training Need Identified 14
Implementation and Monitoring 14

References 14
Appendices 14
Appendix 1: List of Investigative Samples for Medical Termination 16
Appendix 2: Checklist for Babies over 24 Weeks or Babies under 24 weeks Registered as Live Births 17
Appendix 3: Check list for Termination or IUD less than 24 Weeks 19
Appendix 4: List of Investigative Samples for Unexplained Fetal Loss 20
Appendix 5: Information for Obstetricians When Inducing Women/birthing people with Scarred Uterus 22
Appendix 6a: A Simple Guide to the Postmortem Examination 24
Appendix 6b: A Detailed Guide to the Post-Mortem Examination Procedure Involving A Baby 26
Appendix 8: Postmortem Consent Form 34
Appendix 9: Postmortem/Placenta Request Form for Histology 42
Appendix 10 – Information regarding Cytogenetic testing 44
Appendix 11: Mortuary Infection Control Notification Sheet 47
Mortuary 47
Appendix 12: Loss of Early Pregnancy * Consent Form for Funeral Arrangements (< 24/40) 48
Appendix 13: Cremation/Burial Form for Non-Viable Foetal Remains (<24/40) 49

Aims

This guideline covers the management of pregnancy loss from 18 weeks gestation which will be managed on the Labour Ward. Pregnancies that are less than 18 weeks gestation, women/birthing people will follow the gynaecology guideline.

Introduction

Pregnancy loss over 18 weeks can be:

- Unexplained Intra-uterine death (IUD)
- Second trimester miscarriage
- Late termination of pregnancy (TOP) due to fetal anomaly
- Stillbirth/neonatal death

Women/birthing people should be seen as quickly as possible by sensitive staff that are fully informed of the situation and should be assessed on an individual basis taking into account their religious and cultural beliefs.

The Guidance

Initial management of pregnancy loss

If the fetal heart is not heard on auscultation then fetal loss must be confirmed by an ultrasound by the senior obstetrician or the ultrasound department in working hours. If the fetal heart is not located at scanning, the ultra-sonographer will inform the midwife/on call obstetrician. The midwife will find the woman/birthing person a quiet room and inform the Labour Ward obstetric speciality doctor or Consultant to review the woman/birthing person and to discuss management.

- Offer condolences
- Ensure that the woman/birthing person has someone to support her when she receives the news.
- Explain process, method of delivery and analgesia available.
- The midwife can offer bereavement leaflets if appropriate
- Inform patient's named Consultant team or registrar-on-call if team not available
- The doctor will consent the woman/birthing person verbally and document her decisions in the hospital notes.
- The obstetrician should inform women/birthing people that after the diagnosis of late IUD mothers sometimes continue to experience (passive) fetal movement.
- **Mifepristone 200mg** orally is recommended for all medical terminations and for all IUDs. It is a synthetic antiprogesterone, which sensitizes the pregnant uterus to administered prostaglandins such as Misoprostol, Prostin® and Propess®. When used during mid-trimester TOP the procedure is easier and less protracted. It must be taken in the presence of registered practitioner. Temperature, pulse and blood pressure (BP) must be recorded before and two hours after ingestion of tablets. Patients may then go home, but should return to delivery suite at 36-48 hours later but phone first. They will be given the contact numbers to phone triage/Labour Ward in case of any concerns or if contracting.

NB. (Mifepristone) tablets are kept in the drug cupboard on Labour ward along with drug audit forms. The drug can only be used on a named patient basis. The drug must be prescribed, and the audit forms must be filled in by the prescribing doctor and sent to pharmacy. Pharmacy will only replenish supplies if forms are completed.

- Record the patient's information on the high-risk white board in the Labour Ward Office. The hospital notes will be stored in the notes trolley on Labour Ward.
- If the woman/birthing person wishes to wait, ensure that a management plan is clearly documented with a follow up appointment after discussion with the Consultant.
- Women/birthing people contemplating prolonged expectant management after diagnosis of IUD should be advised that the appearance of the baby may deteriorate and the value of a post-mortem may be reduced.

Initial management of a fetocide (>21 weeks gestation)

- If a fetocide for fetal anomaly is necessary (>21+6 weeks gestation), ensure that this has been carried out and that fetal heart asystole is documented 30 minutes post procedure (this procedure is carried out at a tertiary centre). We refer the woman/birthing person to Queen Charlottes Hospital for all our procedures under Fetal Medicine.
- If not, asystole must be confirmed by ultrasound by the maternity unit in which the woman/birthing person is currently being seen.
- It is important to note that the termination commences with the administration of Mifepristone. Medical staff should ensure that the requirements of the Abortion Act are met before prescribing Mifepristone (Ensure that the form (HSA1) or 'blue form' for TOP, as stated in Abortion Act 1967, is signed by two medical practitioners. It may have been signed at the tertiary centre) See Appendix 1 for the checklist for medical TOP bloods and samples.

Timing of delivery

- Usually the woman/birthing person is advised to come in 36-48 hours after Mifepristone
- If the woman/birthing person shows signs of sepsis, pre-eclampsia and placental abruption or premature rupture of membranes they should be strongly advised to take immediate steps for delivery.
- If the woman/birthing person wishes to wait longer than 48 hours for delivery, then bloods should be taken for disseminated intravascular coagulation (DIC) twice weekly.

Process For Delivery

When the woman/birthing person arrives, show her to our bereavement suite (Ocean Pearl) and issue any parking permits needed. Commence checklist appropriate for case (Appendices 2 and 3)

Care Pathway

- All observations are documented in Obs and Assessments on Cerner
- Commence Partogram on Cerner when indicated or when the birthing person reports an increase in regular pain/ contractions
- There is a bundle on Cerner in orders for the correct samples needed called 'Bereavement IUD' you can also refer to appendix 1or 4 for correct invasive tests
- Once contractions start, frequency of maternal observations are recorded as per 'Care in Labour' guidelines on the Partogram
- CTG monitors should **NOT** be used

NB Special caution should be exercised for women/birthing people in the third trimester and for women/birthing people with previous uterine scars: **all such cases will be discussed**

with the Consultant for a plan of care. An experienced midwife should care for women/birthing people in these situations.

Consent to Process

Ensure that the consent has been documented for the procedure and that the risks involved with medical induction and the benefits of this compared to surgical evacuation / hysterotomy / caesarean section. This is especially important in women/birthing people who have had a previous caesarean section where the risks of uterine rupture, though rare are significant.

The potential side effects are: -

- Vaginal bleeding (spotting) – 6%
- Mild pelvic pain – 34%
- Nausea and vomiting – 28% and 20% respectively
- Headaches – 34%

Written consent is required for any fetal samples used for karyotyping and post-mortem, or for taking X rays. Verbal consent must be documented for taking photographs.

Pain Relief in Labour

Women/birthing people should be assessed on an individual basis and offered a choice of effective pain relief depending upon their need. The anaesthetist should be asked to review the woman/birthing person and discuss the different options for pain relief.

- Diamorphine should be used in preference to pethidine.
- Patient controlled analgesia (PCA) - If a PCA is requested please familiarise yourself with the Adult IV Patient Controlled Analgesia Guideline where you will also find the observations that need to be carried out for anyone on a PCA. To document these observations, you will need to add the HDU navigator band to your personal navigator bands on Cerner.
- Epidural, providing an assessment for the risks for DIC and sepsis has been undertaken prior to administering a regional anaesthesia.

Inducing Labour and Delivery

Mifepristone 200mg orally should have been administered 36-48 hours prior to admission (Appendix 5 Information for obstetricians on IOL with CS scar). The following table shows what drugs should now be considered.

	TOP < 27 weeks (no prev CS scar/prev uterine surgery)	**TOP < 27weeks (with CS scar/prev uterine surgery)**	**IUD < 27 weeks (whether prev CS scar or not)**	**Any pregnancy ≥ 27 weeks IUD/TOP**
Misoprostol per vagina	400mcg every 3 hours max 5 doses	100mcg every 6 hours max 5 doses	100mcg every 6 hours max 5 doses	**Consultant decision** 50 mcg every 4 hours max 5 doses (no previous CS) 25-50mcg every 4 hours max 5

				doses (with CS scar)
Prostin or Propess per vagina				**Consultant decision**
Consider mechanical methods for induction of labour e.g. Foley catheter inserted into the cervix and inflated with 30 mls water				**Consultant decision**

Cautions

- No Aspirin and no other NSAID should be given between Mifepristone and delivery because non-steroidal may interfere with the action of prostaglandins
- Asthmatics and patients with chronic obstructive airways disease.
- Patients with cardio-vascular disease.
- **Contraindications -** Patients on long term corticosteroid therapy or with chronic adrenal failure should not have this preparation.

The timing and the dosage of prostaglandin/ misoprostol should be clearly documented.

Midwives may administer the first and subsequent doses of prostaglandin as prescribed by a doctor, providing that the woman/birthing person has no history of previous uterine surgery and is not experiencing excessive pain.

The obstetrician should review the patient 6 hourly until delivery has been achieved.

If the woman/birthing person has not delivered 24 hours after the first dose of prostaglandin, (misoprostol, Dinoprostone (Prostin E2® or Propess®) she must be reviewed by a Consultant with a vaginal examination to make a plan for further management.

Preparation for Delivery

Physiological management of third stage, prepare for potential active management of third stage.

Equipment Required

- Inco pads
- Bed pan moulds
- Delivery pack
- Baby clothes from parents or clothes available from snowdrop if appropriate
- Cold cot, turned on once delivery is imminent
- Deceased Person's Label (this can be prepared in advance and is required for transferring the baby to the mortuary)
- Digital camera, kept in Snow Drop
- Prepared memories booklet and keepsake box (if appropriate/with parental consent)

Prepare for delivery

- Find out their expectations and preferences and whether they want to see the baby or not.
- Discuss their Birth Plan
- Be supportive and aware of individual needs of the people in the room
- Cover the toilet seat with a bedpan mould in case the woman/birthing person goes to the toilet unattended.
- Instruct the woman/birthing person to expect a possible delivery with the urgency to open her bowels or pass urine
- The delivery should be conducted in the same way as a term but handled with appropriate sensitivity.
- If there is a fetal condition that is known not to be compatible, ensure the parents understand that signs of life might be present at birth and the baby could continue to breathe for some time.

Signs of pending delivery

- Increased intensity of pain usually in the lower abdomen and back
- Blood-stained show
- Increased vaginal loss
- Vaginal discomfort / heavy feeling in vagina (due to fetal parts presenting through the dilating cervix)
- Increased anxiety.

Delivery

- Once the baby has delivered, cut and clamp the cord, place the baby on a towel. Depending on the woman's request, the baby's body can be rested either in her arms or in the cold cot (do not place baby in cold cot if born with signs of life)

NOTE: In the case of an acute obstetric event that leads to an intrapartum fetal death, where possible, obtain cord blood or fetal blood for chromosome analysis. Use a lithium blood bottle (orange top) 2mls only.

Pre-viable

- The woman/birthing person should be advised about the potential difficulty in sexing the baby, before she delivers
- If a baby is born alive and subsequently dies, a doctor must attend before and after death to certify the baby has deceased
- Baby's ID band should include time and date of birth and needs to include mother's/birthing person's details with their NHS number

Please refer to www.mbrrace.ox.ac.uk **for criteria for reporting data.**

Management of the third stage

Usually, the baby and placenta deliver together, but occasionally the placenta may be retained. If a dose of prostaglandin is due, it should be given, and if bleeding is not excessive it is reasonable to wait up to 4 hours before undertaking a manual removal of placenta under regional or general anaesthesia. A vaginal examination should be performed before taking the patient to theatre, to be sure that the placenta is not in the upper vagina. These women/birthing people are at high risk of retained products of conception and therefore a low threshold for examination under anaesthesia should be considered.

Observations of maternal pulse, BP and temperature and uterus are done post-delivery as per 'Care in Labour' guideline'

- Doctor to prescribe Cabergoline (as appropriate).
- If Rhesus Negative, ensure that anti-D immunoglobulin is prescribed and given prior to discharge home.
- A follow up postnatal appointment with the Consultant will be arranged by the secretary (ensure the notes are sent to the appropriate secretary to arrange)

Care of the Baby's Body

- In view of the baby's fragility, minimal handling is essential
- Parents can bath their baby if appropriate, the bath is located in snowdrop
- Gently wipe any blood away using cotton wool or gauze soaked with water
- Clinical observation of the body must be documented in the notes.
- The woman/birthing person may choose not to see the baby
- She may want the baby cleaned before viewing

Preparing the baby for viewing:

- If the parent chooses to view the baby, preparation of the baby's body can be done in Snowdrop and ensure the occupancy is noted on the Labour Ward report board
- Present the baby in appropriate clothing depending on size
- Ask the parents if they have any additional keepsakes to keep next to the baby.

Taking hand and footprints:

NB Consent must be obtained for taking prints

- Hand and footprints can be placed in the memory booklet provided

Taking Photographs

Consent must be obtained for taking photographs – document in the woman's notes. Parents can be offered a keepsake box and/or the memories booklet.

Taking photographs:

- **'Remember my Baby'** is a charity that can be contacted to take photographs of babies over 24/40
- The baby should not be photographed naked
- Support the baby's body with a surrounding sheet / blanket. This can help to obtain the best aspect of the baby's features. The baby's head turned to one side can sometimes improve the facial appearance. A hand placed over the outside of the blanket can also help the woman/couple focus on a possible normal feature.

The baby's appearance will alter with the passage of time. To ensure the best image the photographs should be taken as soon as is practically possible.

Blessings/Saying Goodbye

The hospital chaplaincy service can offer the parents a blessing for the baby. See chaplaincy site on the intranet for the help book. They can also support cultural needs in relation to death.
Chaplaincy services are available Monday to Friday, 08:00-16:00. We have Christian and Sikh chaplain cover. In addition, an imam provides voluntary support of Monday, Wednesday and Fridays, and may also be available on other weekdays during office hours

on an on-call basis. Out-of-hours support is not routinely provided; however, the switchboard holds a contact list of volunteer faith leaders who may be available by phone or in person, though in-person support cannot be guaranteed.

Advice for Parents Choosing a Postmortem

Consent needs to be taken by a doctor, or a midwife who has full understanding of the consent form and the pathologist requirements. The person must be trained in taking consent and specifically approved and signed off under hospital/trust/board policies to take postmortem consent/authorisation, or should be deemed to be competent, and should understand the potential benefits to the parents of a postmortem.
A Simple Guide to the Postmortem Examination for Babies – use this guide to provide concise written literature on post-mortem. (**Appendix 6a)**

Parents can have the extended version: **A detailed guide to the postmortem examination procedure involving a baby (Appendix 6b)**

- Advise the parents that the decision does not need to be made immediately; time must be given to them to choose
- Parents should be advised that no specific cause is found in almost half of stillbirths.
- Parents should be advised that when a cause is found it can crucially influence care in a future pregnancy.
- If the parents do not wish to have a full or limited examination, other investigative samples can be offered, i.e. maternal swabs and blood, fetal skin, swabs and placental tissue. These may give some indication as to why the baby died.
- Where a diagnosis of chromosomal abnormality has been confirmed by amniocentesis or chorionic villus sampling, fetal samples do **not** need to be taken.
- If an antenatal diagnostic test has been done ***but results not yet available*****,** fetal samples ***will*** need to be taken (please refer to page 37, Appendix 3).

Consent to a hospital postmortem examination on a baby born in Hillingdon, should be completed: (Appendix 7)

- Photocopy the consent form twice
- One copy in the blue notes
- One copy for the parents to take home
- Original copy to the mortuary with the baby
- **The baby and the placenta are to go to the mortuary together**. Please note that if postmortem has been consented to, the placenta is to be **sent dry** in an appropriate container/ white bucket labelled with the A maternal addressograph with her NHS number should be placed on the side of the container and the lid
- The baby and the placenta will not be transported to John Radcliffe Hospital for post mortem until 'changing your mind' (section 1) has expired
- **For Postmortems** – The member of staff must contact the John Radcliffe Hospital first and book the baby in for PM before scanning in any paperwork for them. The John Radcliffe Hospital mortuary must be telephoned Monday to Friday between the hours of 6.45 am – 1pm. Use the checklist and all the postmortem paperwork which is found in Snowdrop which has the relevant details that they require and the telephone number. If the person is on annual leave or the telephone call is made after 1pm or out of hours contact the mortuary staff directly on 01865 220495 make sure you leave all the information for them that is on your checklist. Telephone the next day to ensure that the paperwork is correct.
- After calling them and the postmortem has been booked. Then scan in and email the histology/placental form, consent form and scan in the genetics form to: mortuary@ouh.nhs.uk; caz.costar@ouh.nhs.uk

- In the case of a neonatal death where a postmortem is being requested to contact the neonatal pathologists to Dr Fowler 01865 220504 or Dr Thomas 01865 222022 to discuss the case.
- If you are sending a baby for post mortem that is an External Only Post Mortem with the placenta - please make sure you complete Section 2 of the consent form as this also relates to blocks and slides taken from the placenta – so if this has not been correctly completed this will delay the post mortem as we then have to chase you to get you to complete this part correctly – If they say NO you must write in the section below their wishes for the blocks and slides from the placenta following the examination.

Parents who are undecided or who have declined the postmortem

- The whole placenta can give an indication of an underlying abnormality complete placenta request form appendix 8.
- The person obtaining consent for postmortem should consider taking x-rays for abnormalities if parents decline a PM
- It is important that the baby is examined, and the examination is documented in the notes. If appropriate a Senior Paediatrician can be consulted.

Placenta Only

Request form to be completed by the midwife.
The whole placenta should be placed in a white specimen pot and should be covered with formalin**.**

- A maternal addressograph with her NHS number should be placed on the side of the container and the lid
- The request form should be placed with the placenta including copies of USS please make sure you have circled parents' consent on disposal of placenta
- The specimen pot is placed in a clear plastic bag and placed in the red box in the sluice ready for collection.
- Please enter all details of request in the diary which is locked in the cupboard with the formalin

Referral for Cytogenetics Analysis of Amniotic Fluid, CVS and Solid Tissue Samples

Under the options for sample, tick the selection box 'Other' and give details in space provided See Appendix 9: Northeast Thames Genetics Service:

- The parents need to consent for the removal of solid tissue samples (placenta, skin)
- A piece of fetal skin, placed in a universal container filled with normal saline
- A piece of placenta, placed in a universal container filled with normal saline
- Please read guidelines attached to request form

The Hillingdon Hospital investigations

- The fetal and maternal swabs can highlight infection
- The biochemistry, haematology and serology can give an indication of maternal medical conditions.

Prepare the baby for transfer to the mortuary

This can be very distressing for parents and therefore must be done away from view.

- Babies must be wrapped in an inco pad first (absorbent side against the baby) as a blanket or towel may stick to the skin. Ensure the sheet is securely wrapped but not overly taped tight. Next, wrap the baby in a blanket, this may be the parents' own blanket if they wish

- ID band needs to be attached to the baby. This must have the baby's delivery date/date of birth and or death written on.
- Fix the completed ‘deceased person’ label to the top blanket
- When the baby is ready for transfer, please contact the porters on **bleep 5844** or phone **extension 53346**
- The porter will bring with them a transport holder from the mortuary (midwife does not need to accompany the baby to the mortuary)
- Place the baby in the transport holder with any personal belongings and clearly document these items in the notes.

Documents required to go with the baby to the mortuary <24/40

If baby is less than 24 weeks and is non-viable.
Midwife to complete Appendix 10 &11 before sending baby to the mortuary. A photocopy should be kept in the notes

Appendix 11: Mortuary Infection Control Notification Sheet must be completed

Documents required to go with the baby to the mortuary >24/40 or <if registered alive birth

REMINDER: If the baby is to have a postmortem, the original postmortem consent form and placenta request form need to go as well

Bereavement office

- Email to be sent to Bereavement office within 24 hours of birth
- Bereavement office issues a stillbirth certificate. Paperwork must be signed by a midwife or doctor provided he/she has been involved in the care of the mother during delivery or has seen the baby’s body.
- The mortuary will not release the baby’s body for post-mortem until the time for parents to ‘Change their mind’ has elapsed (Section 1)
- Once the paperwork is completed by the midwife/doctor the Bereavement Office will contact the family directly with an appointment and issue documents for registering the birth and death of the baby.
- The Bereavement office will also discuss funeral options

Funeral Arrangements

- The Bereavement office will give information for all babies 24/40 or < if registered as a live birth
- Babies born before 24 weeks can be offered a hospital funeral in the form of cremation or they can choose to have a private service with chosen funeral directors at no extra cost.

Hillingdon Hospital offers a free simple cremation service for all non-viable foetuses (babies delivered pre 24 weeks gestation with no signs of life). The Mortuary staff arranges the cremation with a funeral director that has a contract to the hospital.

However the family may choose to organise their own funeral, some funeral directors will do this free of charge. Staff at Hillingdon Hospital cannot recommend any funeral directors. If the patient does not arrange the funeral within 12 weeks of the delivery the hospital will make the arrangements.

The Mother is requested to complete the form Appendix 12 “Loss of Early Pregnancy, Consent form for Funeral Arrangements. The form needs completing on the ward, and sending to the Mortuary with the foetus.

Cremation

The cremation typically takes place in the first week of the month at Breakspear Crematorium, Breakspear Road, Ruislip, HA4 7SJ. The Hospital Chaplain conducts a service starting at 09:30 and parents can choose if they want to attend the service.

The foetuses are kept for one calendar month from the delivery date then included in the next service e.g. delivery 1-1-26, cremation will be 2-2-26. This usually allows enough time for the foetus to be returned if a post mortem has taken place. At the service, the foetus will be in a coffin and there may be other coffins there, and other relatives attending the service. If the parents request an earlier date contact the Mortuary on ex 53431 to check if this is possible.

The foetuses are usually cremated in the afternoon, after the Chapel service. There is an area in the garden allocated to babies and a bench dedicated to Hillingdon Hospital.

This is what is covered in the contract that the hospital has with the funeral director. If this is not acceptable, the relatives must arrange their own funeral.

Private Arrangements – including burials

The hospital does not offer hospital burials. If the parents would like their baby to be buried, they will need to arrange this independently. Please refer them to the guidance above so they can be directed to the appropriate services.

In most cases, it is best for families to contact their own religious advisor or a local funeral director for advice and support. Funeral directors typically do not charge for funerals for children under the age of 18.

This may also be a suitable option for parents who would prefer a cremation but wish to receive their baby’s ashes and have a more personalised service than the standard hospital-provided funeral.

Mortuary staff do not usually have direct contact with the Mother

Neonatal Death

- Live babies that subsequently die will need a birth and death certificate issued by the civic centre
- Complete Initial notification of the death of a child (Appendix 2a).

Discharge

- Routine postnatal check by midwife or obstetrician
- Complete the Maternity Pregnancy Loss checklist found in adhoc on Cerner
- Offer Cabergoline 1 mg as a single dose (lactation prevention) as soon as possible. If she declines, she should be given advice about supportive underwear for when the milk supply comes in.

NB Prescriber must be aware of contraindications for this drug (see BNF)

Postnatal visits:

- Arrange Community/Bereavement Midwife (CMW) visit at discharge

- Health professionals must be aware that the woman/birthing person and her family are at risk of mental health issues and referred for support as required.
- Email the health visitor (send copy of birth notification if there is one ensure **stillbirth is clearly documented**). Cc The Bereavement midwife.
- Ensure parents have a contact number for Bereavement Midwife and Labour ward
- The women/birthing people will receive a follow up appointment with her consultant
- Consultants follow up appointment will discuss the results from post-mortem etc. The appointment will also discuss the chance of recurrence and any specific means of preventing further loss.

REMINDER:
Congenital Malformation diagnosed antenatally will be notified by the specialist midwives. In the event an unknown abnormality is detected after of birth, the paediatric team needs to inform them PHE.NcardrsTv@nhs.net

MBRRACE-UK Perinatal Death Data Collection Form must be completed for all stillbirths from 22+0 weeks and MTOPs from 24+0 weeks gestation. Or any live births born after 20/40 & weigh 400g who die up to 28 days after birth

Associated Guidelines, Policies and Documents

- Medical Management of Missed Miscarriage
- Pregnancy Loss (IUD) and Medical Termination Between 12 and 18 Weeks
- Consent for Histopathological Examination and Disposal of Tissue Following Early Pregnancy Loss

Training Need Identified

Training for midwives is done by a Bereavement Midwife and the relevant guidelines. Obstetricians and Paediatricians receive training on consent for Postmortems. The health professional must be trained in postmortem consent to take consent for postmortems. The refresher training occurs every 2 years.

Implementation and Monitoring
All guidelines, protocols and procedures are available on the intranet. All members of staff have a responsibility to check the Trust's intranet and ensure that they are referring to the most recent issue of the document. The weekly general information bulletin includes notification of publication of new/updated documents.

This clinical guideline, protocol or procedure will be monitored using local and National guidance and via the Trust Incident and Risk Management Policies. If risks/issues are identified further specific monitoring will be considered, for example, clinical audit or risk assessment

References
RCOG (2010) late intrauterine fetal death and stillbirth.
SANDS (2017) Guide for consent takers Seeking consent/authorisation for the postmortem examination of a baby

Appendices
Appendix 1: Investigative samples for medical termination
Appendix 2: Check list for babies < 24 weeks registered as live births
Appendix 3: Check list for babies> 24 weeks and

Appendix 4: Investigative samples for fetal loss <24
Appendix 5: Information for obstetricians when inducing women/birthing people with scarred uterus
Appendix 6a: A simple guide to postmortem (information for parents)
Appendix 6b: A detailed guide to postmortem (information for parents)
Appendix 7: The Hillingdon Hospital post-mortem consent form
Appendix 8: Information for the post-mortem consent taker
Appendix 9: Post-mortem consent form
Appendix 10: Information regarding Cytogenetics + GOSH genetics referral form
Appendix 11: Mortuary Infection Control Notification Sheet
Appendix 11: Loss of Early Pregnancy * Consent Form for Funeral Arrangements
Appendix 12: Pre-Viable Burial and Cremation Form

Appendix 1: List of Investigative Samples for Medical Termination

BABY				
Form	**Hospital – North East Thames (GOSH)**		**Date**	**Signature**
Appendix 10 (Referral form for NE Thames Regional Cytogenetic test)	Fetal skin	Normal Saline in a universal container		
	Placenta	Dry sterile container but in 0.9% sodium chloride if there is a delay		
Appendix 8 FOR PM ONLY	**Hospital – Oxford Radcliffe**			
	Fetus	Dry		
	Placenta	Dry		

MOTHER				
Haematology	FBC	Small lavender x 1		
Transfusion	Group & Screen	Large pink x 1		

All investigative samples need to have the result directed back to the consultant's secretary for follow up appointment

Appendix 2: Checklist for Babies over 24 Weeks or Babies under 24 weeks Registered as Live Births

	Date	Sign
File this form **inside** the patient's notes **(at the front)**		
Named Consultant informed YES/NO Complete incident form – Ref No:		
Parents given an opportunity to see and hold their baby		
Cold cot/cuddle cot used		
Clinical observations of the baby's body documented in notes		
Cabergoline 1mg prescribed and given within 24hrs of delivery		
Memory box offered: **accepted / declined**		
• Photographs: Consent : **accepted / declined** given to parents and/or filed in notes **(please circle)** • Hand & Prints: Consent : **accepted / declined** given to parents and/or filed in notes **(please circle)** • Remembrance book form given:: **accepted / declined** • Bereavement information given on discharge Yes/No		
Religious and cultural beliefs respected **Yes /No**		
Post Mortem: Consent: **accepted / declined**		
Do parents wish to take their baby home (Sand's leaflet Saying Goodbye to your baby p.26) **YES/NO**		
1. PM consent form completed & a photocopy filed in notes 2. PM request form completed (include any relevant scan reports) 3. Placenta request form		
Email to Bereavement Office within 24 hours of delivery Funeral options will be discussed with bereavement office		
Complete MBRRACE form (Leave in front of notes) Inform parents that a clinical review will take place **They will receive a letter before the review from the PMRT Midwife**		

Midwife to sign death certificate in Bereavement Office (Must have seen the baby) If NND needs to be a paediatrician		
Informed parents when baby is going to the mortuary		
Complete 'logbook'		
For all Neonatal deaths: Child Death Overview Panel to be informed by Paediatric team		
Bereavement Midwife mobile 07929797511 Monday-Friday 9-5		
Email Health Visitor (can be done out of hours)		
Discharge to Community Midwife		
Others i.e. Topaz/Over Seas/Mental Health Agencies		
Notes to ward clerk with pink form for consultant appointment attached		
Midwife responsible for completing check list		

Appendix 3: Check list for Termination or IUD less than 24 Weeks

	Date	Sign
File this form **inside** the patient's notes **(at the front)**		
Named Obstetric Consultant informed Complete Datix if applicable Ref No:		
Parents given an opportunity to see and hold their baby		
Clinical observations of the baby documented		
Cold cot used		
• **Memory box offered: accepted /declined** • Photographs: **accepted / declined** given to parents and/or filed in notes **(please circle)** • Hand & Prints: Consent: **accepted / declined** given to parents and/or filed in notes **(please circle)** • Remembrance book form given: **accepted / declined**		
Cabergoline 1mg prescribed and given within 24hrs of delivery		
Religious and cultural beliefs respected		
Funeral options guide offered / given		
Loss of Early Pregnancy * Consent Form for Cremation or Burial completed **plus appendix 13**		
Do the parents wish to take their baby home (refer to the Sand's leaflet Saying Goodbye to your baby p.26) **YES/NO**		
Post Mortem: Consent: **accepted / declined**		
If PM consented: 1. PM consent form completed & a photocopy filed in notes 2. Copy to Parents		
Perinatal Mortality Review Tool information given to parents from 22/40 and over (or less if alive birth)		
Complete MBRRACE form 22/40 and over **or any live births born after 20/40 & weigh 400g who die up to 28days after birth** Inform parents that a clinical review will take place **They will receive a letter from the PMRT Midwife**		
Bereavement information given on discharge **Yes/No**		
Bereavement Midwife mobile 07929797511 Monday-Friday 9-5 Inform GP- this is automatically done through Cerner at discharge		
Inform CM		
Inform HV		
Complete 'logbook'		
Notes to ward clerk with pink form for consultant appointment attached		
Midwife responsible for completing the check list		

Appendix 4: List of Investigative Samples for Unexplained Fetal Loss

BABY				
Form	**Hospital - Hillingdon**		**Date**	**Signature**
Microbiology	Deep ear	Swab		
	Placental	Swab		
Appendix 10 (Referral form for NE Thames Regional Cytogenetic test)	**Hospital – North East Thames (GOSH) Check Criteria before taking sample**			
	Fetal skin	Normal Saline in a universal container		
	Placenta	Dry sterile container but in 0.9% sodium chloride if there is a delay		
Appendix 8	**Hospital – Oxford Radcliffe**			
	Fetus	Dry		
	Placenta	Dry		
	Hospital – Oxford Radcliffe			
Appendix 8	Placenta only	Send whole in formalin		
MUM (All bloods are on IUD bundle on cerner)				
Biochemistry / Haematology	FBC /HbA1C			
	Clotting Lupus anticoagulant (DRVVT)			
	CRP			
Transfusion	Group & Screen Antibodies Kleihauer			
Serology	Anticardiolipin Toxoplasmosis Cytomegalovirus Syphilis (if not done at booking) Tropical infections Parvo Virus			
Microbiology	Midstream urine (culture & sensitivity)			
	High vaginal swab (culture & sensitivity)			

NB: Request for rubella if negative at booking

All investigative samples need to have the result directed back to the consultant's secretary for follow up appointment.

Consider obstetric complications and risk of DIC

Maternal complications	**Hospital - Hillingdon**		Date	Signature
Pre-eclampsia	U&Es, LFTs, urate urine PCR	Large gold x 1 Universal container		
Obstetric cholestasis	U&Es, LFTs Bile salt	Large gold x 1		
Maternal sepsis	Blood cultures			
Diabetes/ GDM	TRBS/ HbA1c	1x grey and 1 x small lavender		

Instructions for placental/fetal skin biopsy

Fig 5: Placental biopsy taken for genetic Image testing (please do not send smaller biopsies than the one illustrated).

Please ensure you take a sample from the fetal side of the placenta only - the fetal side has a smooth surface and is the side of the placenta where the cord is attached - send a 10 mm2 biopsy cut using a scalpel from near the cord insertion site.

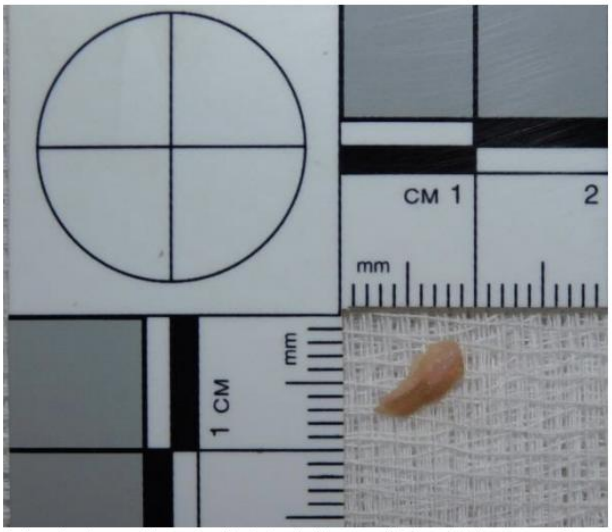

Fig 1: An ellipse of skin taken at Post Mortem for genetic testing (please do not take smaller biopsies than the size shown)

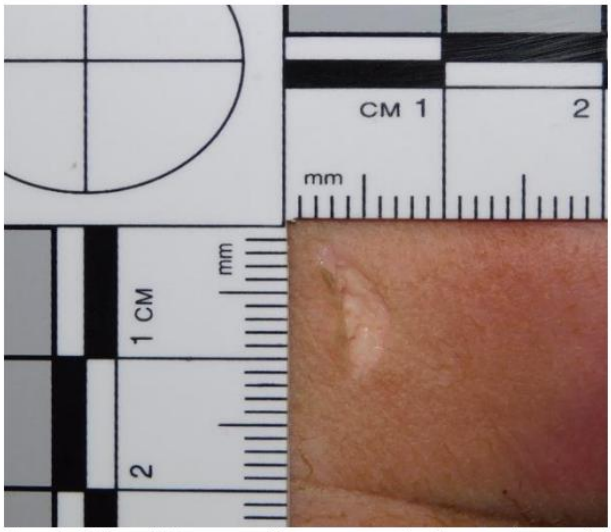

Fig. 2: Site of biopsy after removal of sample sent for genetic testing

Appendix 5: Information for Obstetricians When Inducing Women/birthing people with Scarred Uterus

A randomised controlled trial of oral mifepristone alone (200 mg three times a day for 2 days) was compared with placebo in women/birthing people with an IUFD. Labour occurred within 72 hours in significantly more women/birthing people in the Mifepristone group (63% versus 17%, $P<0.001$).

Use of mifepristone in this context is off label. Mifepristone 600 mg once daily for 2 days can also be used.

A trans- cervical balloon catheter technique was used to induce labour for a small series of 37 women/birthing people with a live fetus and an unfavourable cervix who had previously undergone a caesarean section. There were no complications and 79% achieved vaginal birth.

In another large retrospective study of women/birthing people with one previous caesarean section, induction of labour with mechanical methods resulted in uterine rupture rates (5 in 862, 0.58%) that were significantly lower than with prostaglandins (18 in 1130, 1.59%) and similar to spontaneous labour (51 in 9239, 0.55%).

Mechanical methods of induction might increase the risk of ascending infection in the presence of IUFD.

No studies were found on the safety and effectiveness of induction of labour after IUFD in women/birthing people with a single caesarean section scar. In general maternity care, the RCOG Green-top Guideline on VBAC recommends that women/birthing people should be informed that there is a higher risk of uterine rupture with induction of labour with prostaglandins. The more frequent serious risks of induction of labour with VBAC relate to the fetus, however. In the National Institute of Child Health (NICH) study of 17 898 women/birthing people with a live fetus undergoing VBAC, the maternal morbidity associated with VBAC (including induced and augmented labours) was a higher risk of endometritis (OR 1.62, CI 1.40–1.87), blood transfusion (OR 1.71, CI 1.41–2.08) and scar dehiscence/rupture (0.7%). There was no evidence of an increased rate of hysterectomy or maternal death. Of a subset of 4708 women/birthing people who had had labour induced, 48 had scar problems (1%).

The Society of Obstetricians and Gynaecologists of Canada recommended that misoprostol is contraindicated in women/birthing people with previous caesarean delivery because of a high rate of uterine rupture. A more recent narrative review of induction of labour for late IUFD concluded that misoprostol can be used safely at lower doses for women/birthing people with a previous caesarean (25–50 mcg). Misoprostol is off label for this indication, however, and is not currently marketed in the UK at the doses recommended. These lower doses can be prepared in-house by dissolving a 200-microgram tablet in water.

No studies were found on the safety of induction of labour in women/birthing people with two caesarean births and IUFD. A retrospective cohort database study of 3970 women/birthing people with a live fetus and two previous LSCS compared outcome with those for 20 175 women/birthing people who had undergone a single procedure. Thirty percent of labours were induced in both groups. The chance of successful vaginal birth was almost identical (~75%). The chance of major maternal morbidity, including rupture, was higher in the multiple LSCS group, but the absolute risk remained low: 3.23% overall (rupture 1.8%) versus 2.12% overall (rupture 0.9%, adjusted OR 1.61, 95% CI 1.11–2.33).

Another retrospective multicentre study of 975 women/birthing people with two previous LSCS and 16 915 with one LSCS reported similar results: uterine rupture rates of 0.9% versus 0.7% (P=0.37), hysterectomy 0.6% versus 0.2% (P=0.23) and transfusion 3.2% versus 1.6% (P<0.001). Induction of labour (all types combined) was a significant risk factor for rupture (1% rate for induction versus 0.7–0.9% overall; OR 1.78, 95% CI 1.24–2.56 in univariate analysis; OR 2.71–2.8, 95% CI 1.56–5.22 in multivariate models).

No studies were found into the safety of induction of labour in women/birthing people with three or more
caesarean sections and IUFD. VBAC is not ordinarily recommended for women/birthing people with three previous caesarean sections, previous uterine rupture or upper segment incisions.

In a prospective study of 89 women/birthing people who attempted VBAC after three or more caesareans, including 29 inductions of labour, there were no cases of uterine rupture or of major maternal morbidity, but the upper 95% confidence interval for zero incidence extends to 4% (calculated by guideline developers).

No data were found on the maternal safety of VBAC with an IUFD in the presence of atypical uterine scars.

Appendix 6a: A Simple Guide to the Postmortem Examination

(The extended version of this guide available on request)

A Simple Guide to the Postmortem Examination

We are very sorry that your baby has died. We understand that talking about any practical arrangements and then remembering what has been said may be very difficult at this time. Therefore, we have written this leaflet to give a brief explanation of why the doctors or midwives may ask your permission for a postmortem examination to be carried out.

The information applies to babies who have been born prematurely and who have died either before or after birth.

Please do ask the person who gave you this leaflet or the staff caring for you questions about the postmortem. A more detailed version of this guide is available, and you are welcome to ask for a copy.

Reasons for a Postmortem Examination

If you know why the baby died a postmortem can confirm that the diagnosis or treatment given to your baby was appropriate.

If you do not know why the baby died a postmortem can sometimes provide an answer. Information gained from the postmortem can sometimes be used to care for you in a future pregnancy.

Information gained from the postmortem can sometimes identify an illness that affects other members of the family.

Information gained from the postmortem can help with the discovery of new diseases and treatments.

A Consented Postmortem

You will be asked to sign a consent form before the examination can take place. You can ask for the examination to be restricted.

You can decline to give consent.

What is a Postmortem Examination?

A postmortem involves a careful external examination of the body. This will include the use of X-ray, scans and photographs.

All main organs inside the body can then be examined. These are the brain and the organs in the chest and abdomen.

It may be possible to examine just some of the organs, such as the heart and lungs.
Usually looking at the organs does not give enough information, and it is necessary for samples to be taken and preserved to allow tissue to be examined under a microscope.
These samples taken are very small and are then kept as part of your baby's record. Sometimes it may be essential to examine larger samples but the person giving you information about the postmortem will explain if this is necessary in the case of your baby and also what choices you have about what happens to the samples in the future.

All postmortem examinations are carried out as soon as possible after the death, usual within 2 or 3 days. It may be possible to arrange things more quickly if it is essential to enable a

funeral to take place. Postmortem examinations are carried out by specialist doctors called pathologists who are assisted by technical staff.

It is very important that you feel you can ask any questions you have at any time.

Results of the Postmortem Examination

If you have given consent for the examination of your baby, the consultant who looked after you will offer you an appointment to come back to the hospital and talk through the results. This is usually about 6 weeks after the birth. Very rarely specialist tests can take weeks or months.

Appendix 6b: A Detailed Guide to the Post-Mortem Examination Procedure Involving A Baby

This detailed guide has been written to give you clear information about the postmortem and its potential value.

What is a postmortem examination?
A postmortem is an examination of a body after death (it is also called an autopsy).

Postmortems are carried out by pathologists. Pathologists are doctors who specialise in the diagnosis of disease and the possible identification of the cause of death.

Will I be able to ask questions?
You can ask as many questions as you like.

You may want to discuss the decision about a postmortem examination with the pathologist.

If you would rather not know about certain aspects, please say so.

Why do a postmortem examination?
A postmortem examination can provide information about the illness or other cause of your baby's death. Without a postmortem, the assumed cause of death can be wrong in up to 30% of cases, although it's worth pointing out that even a postmortem examination cannot always provide a reason for the death.

Postmortem examinations can provide information about:

- The cause of death
- The nature of the illness and/or the extent of the disease.
- Conditions that may not have been diagnosed
- The effects of treatments and drugs, and identify any complications or side-effects
- Appropriate planning for future pregnancies and care in pregnancies.
- Diagnose and treatment of conditions other members in the family may have.

Who decides whether a postmortem should take place?
You can ask for a postmortem to be done.

A hospital can request that a postmortem examination is carried out, but this can only be done with your consent.

By law a coroner can order a postmortem to be done. There are three main reasons why a death is referred to the coroner:

- A death has been sudden and unexpected.
- A person has been ill but the doctor confirming the death is not certain why it happened at that time.
- A death has been the result of an accident or unusual circumstances (including deaths following a medical procedure such as surgery).

What if I don't want a postmortem to be carried out?
If the hospital requests a postmortem, and you do not want to agree to a full postmortem examination, you are under no obligation to agree to one.

You might however consider a limited examination.

Usually, it means that only certain parts of your baby's body are examined. However, as the term suggests, this is likely to provide only limited information about the cause of your baby's death or illness.

The doctor or other health professional that discusses the postmortem with you will be able to explain what the options are.

If the postmortems was ordered by the coroner, the coroner does not need your consent to carry out the examination.

When is a postmortem done?
Postmortems are usually carried out within 2–3 working days of the death. The earlier the examination is held the greater the chance of gaining information.

The procedure takes place in a mortuary.

Postmortem paediatric specialists at other hospitals undertake the examinations. Staff should tell you which hospital your baby will be transferred to.

If for religious beliefs you wish to have a funeral within 24 hours, please let the hospital know and the pathologist will try to do the postmortem within this time limit.

If the postmortem was requested by a coroner the coroner's office will normally be able to let you know the date, time and place of a post mortem.

Is anyone but the pathologist present during the post mortem?
The pathologist is helped by other staff usually at least one mortuary technician.

Sometimes medical students, doctors and other health care professionals attend a postmortem examination. This enables there to be quality checks on standards of care as well as training.

If the coroner has ordered the postmortem, as next-of-kin you have the right to be represented at the examination by a medical practitioner.

What happens in a postmortem examination?
The pathologist, working to standards set by the Royal College of Pathologists, will remove and examine all the major internal organs (or those agreed beforehand with you).

Samples of tissue and fluid (such as blood) will be taken for later detailed inspection.

The organs are then returned to the body. The samples of tissue taken for testing are usually kept as part of your baby's medical records.

What happens to tissue samples removed for more detailed inspection?
Although some information can be obtained from looking directly at organs in a postmortem examination, often the only way to understand properly what has happened is to look at part of an organ with a microscope.

Small pieces of organ tissue are removed and placed in cassettes. These samples are usually less than 1 cm and 5 mm thick.

The brain or lung samples will often be slightly larger, approx. 2 cm.

All tissue samples are chemically treated to remove water.
The water is replaced by wax.
The wax hardens creating a tissue **block.**
The tissue block can then be cut into very thin sample slices.
The samples are placed on glass **slides**.
The glass slides can then be examined under a microscope.
More than one section can be cut from one block.
Most blocks and slides are kept securely in special cabinets designed for this purpose.

In babies some organs are very small. Sometimes a tiny organ may unavoidably be taken along with a tissue sample and will form part of the resulting tissue blocks and slides.

Does the hospital keep tissue samples removed during the postmortem?
Yes.

Methods of examining tissues improve year on year.

In cases of genetic disorders, looking back to the tissue of deceased family members may help make a diagnosis in living members of the family.

Tissue blocks and slides may be used in training doctors and other health professionals.

They are also helpful for quality assurance and audit purposes.

Tissues samples may be needed to check on standards in a hospital pathology service.

Will tissue samples be used for general medical research?
Only if you give your permission.

Special committees must approve any research, to make sure it is ethical, and that sufficient consent has been given. Tissue must not be used for research without your agreement.

If you consent, tissue blocks and slides can be used in research. When a new disease or health problem emerges, examination of these stored tissue samples may provide clues about how and why the disease emerged.

What happens to any tissue removed during the postmortem but not prepared for further examination?
When a tissue sample is taken it is often larger than the piece needed for making into a block.

Any tissue left over is usually disposed of immediately, normally via incineration. The tissue sample can be returned to the hospital for the parents to cremate or bury as required.

After tissue slides have been prepared the unused tissue in the block is stored indefinitely.

Only if you give your consent can this residual tissue be used in medical research.

Are photographs taken during the postmortem?
The pathologist will sometimes take photographs, X-rays and CT scans.

These images are usually kept indefinitely as part of your baby's medical records.

They may be used for medical research, education or audit. Any information, which would allow your baby to be identified, will have been removed.

If you object to images being used in this way, you must say so.

Are postmortems on babies carried out in the same way as on adults or older children?
The postmortem procedure on a baby is the same as for an adult or older child, but cases can be more complex.

Genetic testing
Genetic investigations are important if a baby dies during pregnancy or childbirth, to try to find out if the cause is hereditary and if there is a chance of this happening again during a subsequent pregnancy.

Genetic testing is therefore common for babies.

Retention of whole organs
The organs of a baby or young child are very small. Therefore, the whole organ, not just a sample of it, may need to be examined with a microscope.

Sometimes a major organ such as the heart or brain needs to be retained for detailed examination.

You should be aware that if the organ cannot be retained it might be difficult or impossible to reach a diagnosis of why your baby died.

After the examination you can choose to donate your baby's organ for education or research, or you can decide how you wish it to be cared for.

Congenital abnormality
This is a condition that is present from birth, which is a result of hereditary or environmental influences.

These are often more complex in children than in adults and require more extensive investigation.

What happens after the examination has been completed?
If any organs and tissue have been taken in addition to that for tissue blocks and slides, some of the tests on these may take several days or weeks. Hospital staff or the coroner's officer can advise if this is likely, and how long tests may take. You may wish to have the funeral during this time.

Who decides what happens to tissue or organs kept for further examination?

You do, as next-of-kin, other than the tissue samples prepared for examination under a microscope.

You have four options:

- You can ask for the organs and tissue to be returned to the funeral director (or direct to you) once the tests are complete. You will then need to arrange for a separate cremation or burial service for these.

- You can delay the funeral so that organs and tissue may be returned to your baby's body for burial or cremation.

- You can donate tissue samples or organs for use in medical education or research.

- You can ask the hospital to dispose of the organs and tissue in a lawful way. This is usually by incineration.

Can I see my baby's body after the postmortem?
Yes.

After the postmortem the mortuary staff will prepare your baby's body for you to see and hold again, if you wish.

You also have the option of viewing the baby's body at the funeral director's premises.

You should be prepared for there to be changes to your baby's body.

You might want to discuss beforehand with the mortuary staff or funeral director what to expect.

Is the consent form binding?
Unless the coroner has ordered it, the doctors need your consent before they can carry out the postmortem examination.

The consent form acts as a written record of your decision.

It makes it clear to everyone what you have and have not, agreed to.

If you change your mind before the postmortem has taken place you can modify or withdraw your consent – even after signing.

The hospital staff or coroner's officer will tell you how much time you have in which to do this.

What if I get confused about what I'm agreeing to?
The hospital staff should make sure you know enough about the postmortem examination to decide if you wish to give your consent.

They will discuss the alternatives with you.
Although they may recommend a particular option, it is important that you come to your own decision. They will ask you to say whether you have understood the information you have been given.
If you are not sure, don't hesitate to say so.

Can I find out the results of the postmortem?
You will be offered an appointment with the Consultant Obstetrician. Depending on what tests have been requested this appointment is about six weeks after the postmortem.

A copy of the postmortem report will usually be sent to your GP.

In the case of a postmortem ordered by the coroner you should still be able to obtain a copy of the report from the coroner's office. Generally, there is a fee for this, unless an inquest has been ordered.

You should be informed as soon as the coroner's report is available.

Why do hospitals need people to donate tissue and organs for medical research, education, and audit or quality assurance?
Medical education, audit and quality assurance are an integral part of health care and blocks and slides taken for diagnosis may be used in this way.

Tissue blocks and slides are used to train medical students and new doctors.

To help experienced doctors continue to learn about new conditions or treatments.

Doctors training to be pathologists need to watch and learn about postmortem examinations and discuss the findings with an experienced doctor.

Examining tissue is one of the most important ways in which doctors learn about illness and how to treat it.

Research is vital for the progress of medicine.

The majority of research adds just a little understanding to the building of knowledge – occasionally it is ground-breaking.

We ask for your help to continue improving our knowledge and our ability to help other patients.

Can I choose to donate tissue or organs just to certain areas of research?
Yes.

If there is any particular type of research you are worried about, you should discuss this with a member of hospital staff. You do not have to give overall consent but can exclude certain types of medical research.

What happens to organs or tissues donated for medical research, education, and audit or quality assurance?

They cannot usually be returned to you. After they have been used, they will normally be disposed of by incineration. In some cases, they may be used for many years before disposal.

Appendix 7: Notes for the consent taker

2.1 Consent taker training and sources of information

- You should have been trained and specifically approved and signed off under hospital/trust/board
- policies to take postmortem consent/authorisation, or should be deemed to be competent, and
- should understand the potential benefits to the parents of a postmortem (see Section 8).
- You should have observed a postmortem examination (HTA Code B:19), if possible that of a baby.
- You should have read the consent/authorisation policy of your hospital, trust or health board and
- other relevant local policies and guidelines.
- You should be completely familiar with the consent/authorisation form that you will be using.
- You should have read the Sands Booklet Deciding about a post mortem examination: Information
- for parents.
- You should also have read any written information about postmortems for parents produced
- by your hospital, trust or health board.
- You should have read the relevant parts of the Human Tissue Authority's Codes of Practice A and B

(available at www.hta.gov.uk/hta-codes-practice-and-standards-0). For further information *SANDS (2017) Guide for consent takers Seeking consent/authorisation for the postmortem examination of a baby.*

Written information about postmortems should be offered to all parents before you discuss the form with them.

If the parents have a specific request that you are not sure about, contact the **pathologist before the form is completed.**

Make sure that an appropriate time and date are entered in the *Changing your mind* section at the beginning of the form, and the parent(s) understand what to do if they change their minds. The postmortem should not begin unless this section is completed. **It is your responsibility to ensure that, if the parent(s) change their minds, they will be able to contact the person or department entered on this form.** If the parents do not want a copy of the form, they should still be given written information about changing their minds.

Write the mother's or the baby's hospital number in the box at the foot of each page of the form. For a baby who was born dead at any gestation use the mother's hospital number; for a baby who was born alive use the baby's hospital number.

Sections 2 and 3: Tissue samples (2) and genetic material (3) If the parents do not want tissue samples or genetic material kept as part of the medical record, explain the different options for disposal (below) and note their decisions in the relevant section.

If disposal is requested, it will usually take place only after the full postmortem report has been completed. The options are disposal by a specialist hospital contractor; release to a funeral director of the parents' choice for burial; or release to the

parents themselves. For health and safety reasons, blocks and slides cannot be cremated. Genetic material is normally incinerated.

Send the completed form to the relevant pathology department, offer a copy to the parent(s), and put a copy into the mother's (for a stillbirth or miscarriage) or the baby's (for a neonatal death) medical record.

Record in the clinical notes that a discussion about the postmortem examination has taken place, the outcome, and any additional important information.

Section 5:

Possible further examination of one or more organs Very rarely, it may be recommended that an organ is kept for more detailed examination after the baby is released from the mortuary. In this case, the form *Consent to further examination of organs for diagnostic purposes* should be completed, as well as this form.

If you already know that this is recommended, discuss it with the parents and also explain how it might affect funeral arrangements. If they consent, complete the form *Consent to further examination of organs for diagnostic purposes* now, and staple the two forms together. Record the consent in the *Notes to Sections 1 and 2* on this form.

If the pathologist recommends further examination after the postmortem has begun, they will contact you or the unit. The parents should then be contacted as soon as possible to discuss their wishes and to explain how keeping the organ might affect funeral arrangements. If they consent, the form *Consent to further examination of organs for diagnostic purposes* should be completed and copies distributed as above. A note should be added to the medical record that consent was given, including how it was given (face-to-face, email, etc).

5. Sands Guide for consent takers Jan 2013.pd .2017.pdf (hta.gov.uk) is available at

https://archive.hta.gov.uk/sites/default/files/5._Sands__Guide_for_consent_takers_Jan_2013.pd_.2017.pdf

Section 6:

Parental Consent

Section 7:

Please fill as 'YES' or 'NO'

Consent for transfer

Witness – please list staff midwife

Addressogram – please use mother's hospital sticker

Appendix 8: Postmortem Consent Form

(Please complete this form) and email to caz.costar@ouh.nhs.uk and mortuary@ouh.nhs.uk

Your wishes about the postmortem examination of your baby

Mother	**Baby**
Last name	**Last name** (Mother's Surname)
First name(s)	**First name(s)** (If baby has no name; please write 'BABY')
Address	**Date of birth**
	Date of death (if liveborn)
Hospital no.	**Hospital no.** (if liveborn)
NHS no.	**NHS no.** (if born over 24 weeks)
Date of birth	**Gender** (if known)
Consultant	**Consultant** (same as Mother's)
Father/Partner with parental responsibility	**Address** (if different from the mother's)
Last name	
First name(s)	
Preferred parent to contact, tel. no.:	
Other, eg, religion, language, interpreter **……..……..**	

How to fill in this form:

- **Please show what you agree to by writing YES in the relevant boxes. Write NO where you do not agree.**
- Record any variations, exceptions and special concerns in the Notes to the relevant section or in Section 5.
- Sign and date the form. The person taking consent will also sign and date it.

Changing your mind

After you sign this form, there is a short time (24 hours) in which you can change your mind about anything you have agreed to.

If you want to change your mind, you must contact:

[Name, department] ……[01895279463 – labour ward]

before [time] ……... on [day] …… [date]

Please be assured that your baby will always be treated with care and respect.

Section 1: Your decisions about a postmortem examination *Select **one** of these 3 options.*

A complete postmortem This gives you the most information. It includes an external examination, examining the internal organs, examining small samples of tissue under a microscope, and taking x-rays and medical photographs. Tests may also be done for infection and other problems, and the placenta may also be examined.

If you think you may have another baby in the future and are worried that the problem might occur again, a complete postmortem is the best way to try to find out.

☐ **I/We agree to a complete postmortem examination.**

OR

A limited postmortem This is likely to give less information than a complete postmortem.

A limited postmortem includes an external examination, examining the internal organs in the area(s) of the body that you agree to, examining small samples of tissue under a microscope, and taking x-rays and medical photographs. Tests may also be done for infection and other problems, and the placenta may also be examined.

☐ **I/We agree to a limited postmortem examination.**

Please indicate what can be examined:

☐ **abdomen** ☐ **chest and neck** ☐ **head**

other .....…….

OR

An external postmortem This may not give any new information.

An external postmortem includes a careful examination of the outside of the baby's body, x-rays and medical photographs. The placenta may also be examined.

☐ **I/We agree to an external postmortem examination.**

Section 2: Tissue samples *Only if you consent to a complete or limited postmortem **(if declined section 1; please write 'NO' in box for section 2)***

With your agreement, the tissue samples taken for examination under a microscope will be kept as part of the medical record (in small wax blocks and on glass slides). This is so that they can be re-examined to try to find out more if new tests or new information become available. This could be especially useful if you think you may have another baby in the future.

☐ **I/We agree to the tissue samples being kept as part of the medical record for possible re-examination.** If consent is **not** given, you must note below what should be done with the tissue samples. If they say NO, you must write in the section below their wishes for the blocks and slides from the placenta following examination.

Notes to Sections 1 and 2 if required (See Section 8 Item 6 for more Information)

Please note that blocks and slides cannot be sent to the crematorium.

(NB – please note if the patient would like blocks & slides to be returned with baby – this may take up to 8-12 weeks)

……

…….....

Section 3: Genetic testing

To examine the baby's chromosomes or DNA for a possible genetic disorder or condition, the pathologist takes small samples of skin, other tissue and/or samples from the placenta (afterbirth). With your agreement, this material will be kept as part of the medical record so that it can be re-examined to try to find out more if new tests or new information become available. This could be especially useful if you think you may have another baby in the future.

(Please see Appendix 9 for GOSH referral criteria)

☐ **I/We agree to genetic testing of samples of skin, other tissue and/or the placenta.**
If samples should not be taken from any of these, please note this below.

☐ **I/We agree to the genetic material being kept as part of the medical record for possible re-examination.**

See Section 8 Item 6 for more information.

Notes to Section 3 if required

……

……

…….....

Section 4: Keeping tissue samples for training professionals and for research

Section 4 covers additional separate consent that you may decide to give. It will not affect what you have already agreed to above, what is done during the postmortem, or the information you get about your baby's condition, but it may be helpful for others in the future.

With your agreement, the tissue samples may also be examined for quality assurance and audit of pathology services to ensure that high standards are maintained.

☐ **I/We agree to the tissue samples being kept and used for quality assurance and audit.**

Tissue samples, medical images and other information from the postmortem can be important for training health professionals. Identifying details are always removed when items are used for training.

☐ **I/We agree to anonymised tissue samples, images and other relevant information from the postmortem being kept and used for professional training.**

Tissue samples, medical images and other relevant information from the postmortem can also be useful in research into different conditions and to try to prevent more deaths in the future. All research must be approved by a Research Ethics Committee.

☐ **I/We agree to tissue samples, images and other relevant information from the post mortem being kept and used for ethically approved medical research.**

You can withdraw consent for any of the above at any time in the future. To do so, please contact the hospital and ask for the histopathology department.

Optional section on retaining organs for the Sands *Postmortem consent form*

Section 5: Keeping one or more organs for diagnostic purposes

In most cases, all the organs will be returned to your baby's body after the postmortem examination. But occasionally the doctors may recommend keeping one or more organs for longer, to carry out further detailed examination to try to find out more about why your baby died. This might take some weeks and so could affect the timing of your baby's funeral. The person who discusses the postmortem with you will tell you if it is likely.

(NB – if USS discloses any particular organ anomaly; please consent here for a detailed postmortem of the particular organ)

☐ **I/We agree to further detailed examination of the organ(s) specified below:**

☐ **Any organ**

☐ **The following organ(s)**

…….

If you agree to further detailed examination, you also need to decide what should be done with the organ(s) after the examination:

☐ **I/We want the hospital to dispose of the organ(s) respectfully as required by law.**

☐ **I/We want the organ(s) returned to the funeral director we appoint for separate cremation or burial.**

☐ **I/We want to delay the funeral until the organ(s) have been returned to my/our baby's body.**

Alternatively, after the further detailed examination, you may decide to donate the organ(s) for one of the following purposes:

☐ **I/We agree to donate the organ(s) to be used to train health professionals.**

☐ **I/We agree to donate the organ(s) to be used for ethically approved medical research.**

If you agree to donate one or more organ(s), they will be respectfully cremated as required by the Human Tissue Authority when they are no longer needed.

If you change your mind about this donation at any time in the future, and want to withdraw your consent, please contact the hospital and ask for the histopathology department.

Notes to Section 5 if required

(If any further details/information please document here)

……..

.....

……..

.....

……..

.....

Section 6: Parental consent

☐ **I/We have been offered written information about postmortems.**

☐ **I/We understand the possible benefits of a postmortem.**

☐ **My/Our questions about postmortems have been answered.**

(NB both signatures not required; either mother or fathers is acceptable)

Mother's name …….. **Signature**

…….

Father's/Partner's name …….. **Signature**

…….

Date ……**Time**

……..

Section 7: Consent taker's statements *To be completed and signed in front of the parents.*

☐ **I have read the written information offered to the parents.**

☐ **I believe that the parent(s) has/have sufficient understanding of a postmortem and (if applicable) the options for what should be done with tissue and organs to give valid consent.**

☐ **I have recorded any variations, exceptions and special concerns.**

☐ **I have checked the form and made sure that there is no missing or conflicting information.**

☐ **I have explained the time period within which parents can withdraw or change consent and have entered the necessary information at the beginning of this form.**

Name ……... **Position/Grade**

……

Department ……. **Contact details (Ext/Bleep)**

….. **Signature** …… **Date** ……..

Time ……..……..

Interpreter's statement (if relevant)

☐ **I have interpreted the information about the post mortem for the parent(s) to the best of my ability and I believe that they understand it.**

Name …… **Contact details**

…….……

Signature ……. **Date** ……..

Time ……..……..

Consent for Transfer of a baby for Postmortem Examination at the Pediatric Pathology, John Radcliffe Hospital, Oxford

I, the parent of …….. have given consent for the post mortem examination of my baby by the Department of Paediatric Pathology at the John Radcliffe Hospital, Oxford under the supervision of a Consultant Paediatric Pathologist. I understand that my baby will need to be transferred to Oxford for the examination but every effort will be made to return my baby to The Hillingdon Hospital as soon as possible.

Signed
…….....
...……

Please Print Name
…….....……

Relationship to
deceased…….....……
….

Date
…….....
.....….

Signature of Witness …….....……. Please print name
…….....……...…….

Date……....
.....….....…

This form is to accompany the Consent for Post Mortem Form and to be sent with baby to The Hillingdon Hospital Mortuary

Addressograph

Appendix 9: Postmortem/Placenta Request Form for Histology

PAEDIATRIC PATHOLOGY CONTACT INFORMATION	
DR D FOWLER	(01865) 220504
DR CM BOWKER	(01865) 222022
SECRETARY	(01865) 221246
MORTUARY OFFICER	(01865) 220495
LABORATORY	(01865) 220492
FAX	(01865) 851106

AUTOPSY REFERRALS – *BEFORE* SENDING THE CASE ALWAYS CONTACT THE DEPARTMENT TO FOREWARN US AND RELAY ANY IMPORTANT INFORMATION.

FOR LABORATORY USE

LABORATORY NUMBER:

DATE RECEIVED:

PATHOLOGIST:

NOTES:

PLEASE REMEMBER TO INCLUDE THE PLACENTA!

MOTHER'S DETAILS

HOSPITAL NO		**ADDRESS**	
NAME			
PREV SURNAME		**CONSULTANT**	
D.O.B		**WARD**	
LMP		**HOSPITAL**	
EDD			

SPECIMEN / REQUEST

IS THE REQUEST FOR EXAMINATION OF:

- ☐ A STILLBORN / FOETAL DEATH?
- ☐ A NEONATAL / INFANT DEATH?
- ☐ THE PLACENTA ONLY?
- ☐ OTHER: ……

DATE: ……

RELEVANT CLINICAL DETAILS AND HISTORY

IUD: Yes/No

Scans:

Parity:

Antenatal Complications:

Intrapartum Complications:

PAST OBSTETRIC HISTORY

YEAR	PLACE	SEX	WEIGHT	GESTATION	DELIVERY	COMPLICATIONS	OUTCOME

GENETICS

- ☐ TAKEN
- ☐ NOT TAKEN:

COMPLICATIONS IN PRESENT PREGNANCY

THREATENED	Y /	**GROWTH RESTRICTION**	Y / N
HYPERTENSION	Y /	**OTHER (DETAILS BELOW)**	Y / N
POLYHYDRAMNIOS	Y /		
OLIGOHYDRAMNIOS	Y /		
APH	Y /		

SUMMARY OF PRESENT DELIVERY

(SUMMARY OF COMPLICATIONS, DELIVERY ETC):

	DATE	**TIME**
FETICIDE *(if applicable)*		
MEMBRANE RUPTURE		
1ST STAGE		
2ND STAGE		
DELIVERY		

BABY / FOETUS	
NAME *(Baby, Mother's surname)*	**HOSPITAL NO** *(if live birth)* .....
GENDER *(if known – please write 'unknown' if not known)*	**PAEDIATRICIAN** *(if applicable)* .....
DOB	**ESTIMATED DATE OF DEATH** .....
WEIGHT AT DELIVERY	**ESTIMATED TIME OF DEATH** .....
GESTATION AND/OR AGE	**FATHER'S NAME** *(if different)*

APPEARANCE	PROVISIONAL DIAGNOSES
BABY / FOETUS / PLACENTA ☐ FRESH ☐ MACERATED ☐ VERY MACERATED	

QUESTIONS FOR THE PATHOLOGIST	PLEASE INCLUDE:	
	COPIES OF THE ULTRASOUND SCAN REPORTS	
	COPIES OF ALL GENETICS RESULTS	
	THE PLACENTA	
	POST MORTEM CONSENT FORM	

ABNORMALITIES / ANOMALIES	
PLEASE GIVE DETAILS OF ANY ABNORMALITIES *(and/or attach copies of the prenatal diagnosis scan / genetics reports)*	
FOR NEONATAL DEATHS ONLY	
NEONATAL COURSE: Brief summary of the neonatal course	**DEATH CERTIFICATE** *(clinical cause of death*

Do the parents consent to disposal of placental tissue as per Hillingdon Hospital protocol? Yes/No

CONTACT DETAILS OF MEMBER OF STAFF COMPLETING THIS FORM			
NAME	.....	**DATE**	.....
SIGNATURE	.....	**STATUS**	.....
TELEPHONE NO	01895279463 (LABOUR WARD)	**BLEEP**	.....

Appendix 10 – Information regarding Cytogenetic testing

(Please see form below if patient meets criteria for

The new eligibility criteria allow for cytogenetic testing for:

1. Stillbirth or IUD after 28 weeks gestation in the absence of other likely causes

2. Fetal loss, termination of pregnancy or miscarriage accompanied by additional features suggestive of chromosome abnormality

   Please note that indicators of chromosomal abnormality do not include:

- Anencephaly
- Preterm delivery/fetal demise due to SROM/placental abruption/twin pregnancy in the absence of any fetal anomaly
- IUD in the absence of fetal anomaly

UKAS MEDICAL 7883

Rare & Inherited Disease Genomic Laboratory
Great Ormond Street Hospital for Children NHS Foundation Trust
Level 5 Barclay House, 37 Queen Square, London WC1N 3BH

NHS North Thames Genomic Laboratory Hub

Please note that samples received with incomplete forms will not be tested

SURNAME:	**FIRST NAME:**	**LAB REF:**	
		Date received (Lab)	
DATE OF BIRTH:	**NHS NUMBER:**	**DATE/TIME COLLECTED**	**SAMPLE TAKEN BY:**
SEX	**GENETICS /HOSPITAL NO**	**REFERRING CONSULTANT** (Full name required)	
PATIENT ADDRESS & POSTCODE		**DEPARTMENT** (required)	**SUBMITTER ID (GOSH Link)**
GP NAME & ADDRESS	**NHS** ☐ **PRIVATE** ☐	**Hospital** (required)	
NHS.NET EMAIL / CLINIC CONTACT NUMBER			

PRENATAL TESTS

NHS patients will routinely be tested in line with NHS England and JCGM prenatal testing guidelines.

Amniotic Fluid ☐ **Chorionic Villus Biopsy** ☐ **Fetal blood** ☐

- ☐ **QF-PCR ONLY**
- ☐ **QF-PCR + MICROARRAY – please give details of scan anomalies below**
- ☐ **QF-PCR +TESTING FOR FAMILIAL GENETIC VARIANT- please give details**
- ☐ **QF-PCR + NON-FAMILIAL GENETIC TESTING– please discuss with laboratory before sampling.**

Down's screen risk	Size of NT	Gestation at NT measurement	Gestation / LMP
		.....	

REASON FOR REFERRAL - ultrasound anomalies/previous genetic investigations in family.

SOLID TISSUE TESTS (Pregnancy/Fetal Loss)

- ☐ **fetal demise after 28 weeks / stillbirth**
- ☐ **third or subsequent consecutive miscarriage**
- ☐ **pregnancy loss before 28 weeks with clinical indication of chromosome abnormality** (details **MUST** be supplied, please use box above)

Note: for any other referral reason please indicate funding source

SAMPLE TYPE: **SEX (IF KNOWN):**

If products of conception or a placental sample need to be returned for sensitive disposal please ensure that this is clearly indicated on the referral form and that an appropriate consent form is attached.
Samples containing fetal tissue will be returned for sensitive disposal. Any samples not returned will undergo disposal organised by the laboratory.

In submitting the sample the clinician confirms that consent for testing and DNA storage has been obtained.
Please use the alternative request form (available on our website – see overleaf) for postnatal referrals

INSTRUCTIONS:

Specimen	Quantity	Container and Actions Required	To arrive in lab
Amniotic fluid	15-20ml*	Universal container Email/ Fax laboratory specimen is being sent	Same day, by 5pm
Chorionic villus biopsy	15-20mg*	Universal container containing 0.9% w/v heparinised saline Email/ Fax laboratory specimen is being sent	Same day, by 5pm
Skin biopsy (live patient)	Skin punch 2mm^3, full thickness	Universal container. Send in sterile 0.9% saline if possible, dry if not. Email/ Fax laboratory specimen is being sent	Same day, by 5pm
Fetuses	N/A	**Fetuses (24+ week gestation by date or scan) will not be accepted by this laboratory**	N/A
Fetal skin biopsy (post-termination/post-mortem)	1cm^3 skin biopsy, full thickness	Universal container. Send dry if possible but in sterile 0.9% saline if delay anticipated	Same day
Products of conception (cord/chorionic villi/cord/foetal tissue)	N/A	Universal container. Send dry if possible but in sterile 0.9% saline if delay anticipated	Same day
Placental biopsy at cord insertion site	1cm^3 with chorionic villi or placental membrane	Universal container. Send dry if possible but in sterile 0.9% saline if delay anticipated	Same day
Fetal blood samples or Cordocentesis samples from ongoing pregnancies with multiple congenital abnormalities (MCAs) and from TOPs with MCAs or IUDs	0.25-2ml	Send in an EDTA blood tube. N.B. a maternal blood sample (2ml in an EDTA blood tube) should be sent if the sample was obtained in utero.	Same day

*Testing may be compromised if a sub-optimal sample is received and may result in a delayed or failed result.

Please email patient details of any pre-booked prenatal sample or skin biopsy for testing (details below).

Samples can be stored and sent at room temperature on the same day, or stored in a fridge overnight. Small samples such as skin biopsies should be placed in sterile saline if being sent the following day.

Sample must be labelled with:

- Patient's full name (surname and given name)
- Date of birth and NHS number
- Referring Hospital Number
- Referring Hospital and full name of referring Clinician (not initials).
- It is desirable to have the date and time sample was taken

Samples MUST BE PACKAGED ACCORDING TO UN PACKING REQUIREMENT 602 and be sent to the following address:

Rare & Inherited Disease Laboratory
Specimen Reception
Level 5, Barclay House
Great Ormond Street Hospital
37 Queen Square
London WC1N 3BH
Tel: 020 7829 8870 Email: genetics.labs@gosh.nhs.uk or gosh.geneticslab@nhs.net

For details of all referral criteria and policies please see our website: www.labs.gosh.nhs.uk/laboratory-services/genetics

For Lab Use Only

Appendix 11: Mortuary Infection Control Notification Sheet

(MUST ACCOMPANY BABY TO MORTUARY AT ALL GESTATIONAL AGES)

Name of Deceased:……

(addressograph label if possible)

Address:……...

Hospital Number:……..……...

Date and Time of Death:…….

Ward:……..

Contact No.:……

Does the deceased have any of the following infections?
DO NOT CIRCLE SPECIFIC INFECTION

- **Meningococcal septicaemia**
- **Tuberculosis**
- **Hepatitis B, C and non-A non-B**
- **HIV/AIDs**

YES / UNKNOWN
(PLEASE CIRCLE)

Rare diseases to Hillingdon: Plague, Rabies, Coronavirus which include SARS, MERS and COVID-19', Smallpox, Viral Haemorrhagic fever (inc. yellow fever), Invasive group A streptococcal infection, VCJD.

If circled **YES** a body bag **IS** necessary according to Hospital policy 45.

The mortuary staff will phone on their next working day to confirm the specific infection

Please tick where appropriate:

- The body is bagged because of **infection**. ☐
- The body is bagged because of **leaking**. ☐

The doctor who is certifying the death or the nurse verifying the death must sign this form.

Signed…….

Print Name…….

Mortuary
Ex 53431

This form **MUST** come to the mortuary with the deceased.

Please ensure the deceased has at least **ONE** ID band on and is suitably clothed (e.g. a shroud/gown).

Appendix 12**: Loss of Early Pregnancy * Consent Form for Funeral Arrangements (< 24/40)**

Name of Mother: ……..……..

(Addressograph label if possible)
Address: ……..……..……...

Patient Number: ……..……

*early pregnancy, less than 24 weeks gestation

Delivery date: ……..……**Gestation:** ……..……

Is a Post-mortem requested? ……..……..

If yes include completed Consent, Request forms and placenta.
Do you agree to disposal of the placenta as per Hillingdon Hospital protocol YES / NO
If NO, the placenta will be reunited with the baby for the funeral. NB: this can take about 6 -12 weeks.

PLEASE TICK ONE OPTION IN RELATION TO FUNERAL OPTIONS BELOW

- ❑ I wish to make private funeral arrangements for the baby – (*once decided, please ask your chosen funeral director to contact the Mortuary via switchboard on 01895 238282)*
- ❑ I would like the baby to be included in the communal hospital cremation service *(please note, hospital burials are not offered)*
  - ❑ I **do want** to be informed of the date and time of the cremation, as I **may want** to attend
  - ❑ I **do not want** to be informed of the date and time of the cremation, and I will **not** be attending

If you have chosen a hospital cremation and would like to be informed, our Pastoral Support team will be in contact on the phone number you provide below to give further details

If you need any further guidance after deciding your funeral arrangement option, please contact our Pastoral Support team on 01895 279433

If you do not inform us your decision about the funeral arrangements by 12 weeks and we have been unable to contact you, the hospital will make the necessary arrangements.

Print name:…… Signature……

Date: …… Best contact number:……

Midwife/nurse/doctor name:…….. Signature:…….

Contact number:……

This form must be sent to the Mortuary with the baby.
Please provide a copy to the patient.

Appendix 13: Cremation/Burial Form for Non-Viable Foetal Remains (<24/40)

This form is to be sent to the mortuary with the baby.

Hillingdon Hospital Mortuary
Pield Heath Road
Uxbridge
UB83NN
Telephone: 01895 279431

CREMATION/BURIAL FORM FOR NON-VIABLE FOETAL REMAINS

Information to be completed by Doctor / Midwife.

Mother's name: …….

Mother's address: …….

……..

Delivery date: ……

Number of weeks gestation: …….....

I certify that I have examined the pregnancy remains of the above mother and there is no reason for further examination.

Doctor/Midwife's signature: …….

Name (block capitals): ……

Date: …….....

Registered qualifications: ……